Two-hour urgent community response

Forecast for September 2026 · data through August 2026

The latest Two-hour urgent community response result was 85.5%. For the next release, it is expected to remain broadly stable at 85.9%. Current-level seasonal and underlying-trend effects are small or offsetting, leaving the outlook broadly stable. The current projection is on trajectory for 70% national standard. Across providers, sustained favourable signals outnumber adverse ones (15 to 8).

LATEST · AUGUST 2026	FORECAST · SEPTEMBER 2026	ACTUAL
85.5%	85.9%	—
0.6pp below forecast	80% expected range 85.2%–86.6%	Not yet released

On trajectory · 70% national standard

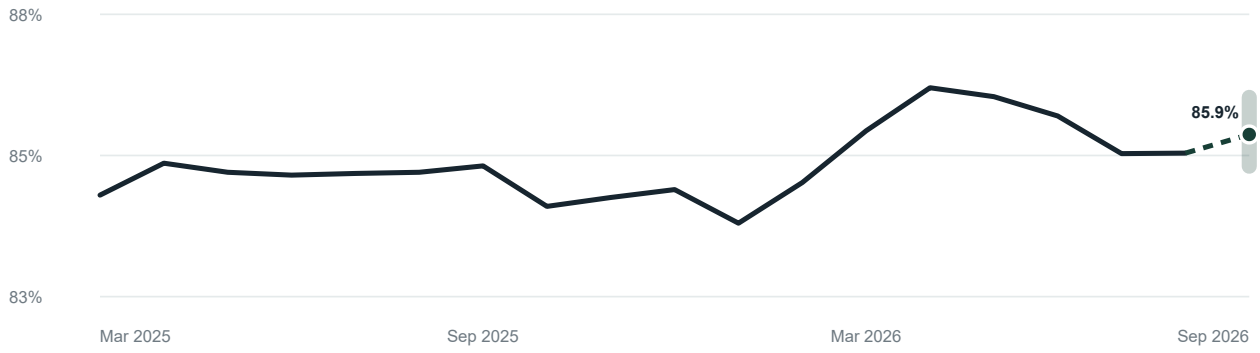
WHAT CHANGED?

Two-hour urgent community response was broadly unchanged in the latest month. The current imported series does not contain a reconciled activity denominator, so this page describes the movement but does not attribute it to demand or throughput.

Monthly	Annual	Volume breakdown
+0.0pp	+0.4pp	Not available

Recent performance and next release

The vertical band is the 80% expected range.



Longer-term outlook and milestones

Descriptive projection; not forced to meet targets.



Benchmark: 70% national standard. 16.0pp above milestone

Data: NHS England two-hour urgent community response. **Targets:** Two-hour urgent community response standard

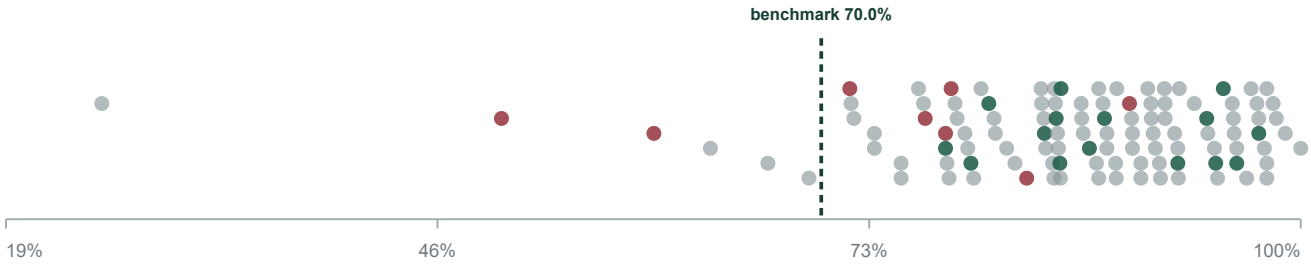
Method: Backtest-weighted ensemble of current-level seasonal movement, damped annual drift and a damped recent seasonal trend; at least 2 components are required. The final fit uses 53 consecutive months. Accuracy is tested on 18 rolling one-month-ahead forecasts. The 80% range uses empirical historical error quantiles from 18 forecast errors. Weights use inverse rolling RMSE and are shrunk toward equal weights. The pre-release commentary compares the current-level seasonal component with the combined forecast. Provider distribution and trajectory watch are shown on page 2.

Two-hour urgent community response

Distribution and trajectory watch · August 2026

Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.

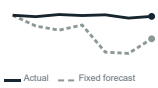


Trajectory watch

Six actual months are compared with one forecast path fixed before that window. Both lines begin at the same observed value. A low performer can be improving, while a high performer can be deteriorating. Status: Actual performance is compared with the monthly path forecast six months earlier. The displayed figure is the average gap across those six months.

83	15	8	60
providers assessed	sustained favourable	sustained adverse	without an active signal

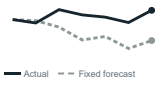
Largest sustained favourable moves

LINCOLNSHIRE COMMUNITY HEALTH SERVICES NHS TRUST
Latest 86.8% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


49.0pp
BETTER THAN FIXED PATH

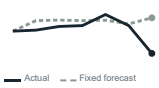
MANCHESTER UNIVERSITY NHS FOUNDATION TRUST
Latest 96.0% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


29.1pp
BETTER THAN FIXED PATH

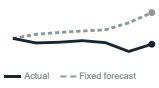
STOCKPORT NHS FOUNDATION TRUST
Latest 92.3% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


23.5pp
BETTER THAN FIXED PATH

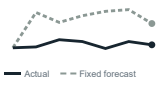
Largest sustained adverse moves

WHITTINGTON HEALTH NHS TRUST
Latest 50.0% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


11.3pp
WORSE THAN FIXED PATH

SOUTH TYNESIDE AND SUNDERLAND NHS FOUNDATION TRUST
Latest 76.5% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


9.4pp
WORSE THAN FIXED PATH

DERBYSHIRE COMMUNITY HEALTH SERVICES NHS FOUNDATION TRUST
Latest 82.9% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


9.3pp
WORSE THAN FIXED PATH

Trajectory-watch rule. Six actual months are compared with the corresponding path fixed before the six-month window. A signal requires a material average gap, at least five of six months in the same direction, and the latest month confirming that direction. These are screening prompts—not league tables, causal estimates or judgements of provider quality.